

WEIGHT LOSS

Patient Screening & Candidacy

Who qualifies, who does not, and why the documented decline matters as much as the approval

DOCUMENT WL-003	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and intake staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This is the gatekeeping protocol. It defines who is a candidate for pharmacologic weight management in this practice, who is excluded, who must be referred, and how each decision is recorded. A program judged after the fact is judged largely on who it declined.

DOCUMENT THE NO

Approvals are easy to evidence — there is a prescription. Declines usually leave no trace, which means a program that screens carefully looks identical on paper to one that screens nobody. Record every decline, the reason, and the referral made.

2 Candidacy Criteria

Criterion	Standard
Body mass index	BMI thresholds for weight-management pharmacotherapy, with a lower threshold where a weight-related comorbidity is present. Your medical director sets the practice thresholds in line with current labeling and guidance.
Measured, not reported	Height and weight measured in the practice at baseline and at each visit. Self-reported figures are not acceptable for a candidacy decision.
Comorbidity assessment	Hypertension, dyslipidemia, type 2 diabetes, obstructive sleep apnea, and cardiovascular disease identified and recorded.
Prior attempts	History of previous weight-management efforts, including prior pharmacotherapy and bariatric surgery.
Realistic goal	A goal agreed and recorded in the patient's own words, with the expected trajectory and its limits explained.
Capacity to engage	Willingness and ability to attend follow-up, complete monitoring, and self-administer or attend for administration.

3 Exclusions & Referrals

Absolute exclusions

- Personal or family history of medullary thyroid carcinoma, or MEN2.

- Known hypersensitivity to the agent or its excipients.
- Pregnancy, or actively trying to conceive.
- Breastfeeding.
- BMI below the practice threshold with no qualifying comorbidity — including patients who are already at a healthy weight and requesting therapy for cosmetic reasons.
- Age below the threshold set by the medical director for this practice.

Requires evaluation or co-management before proceeding

Finding	Action
History of pancreatitis	Prescriber assessment required. Many practices exclude entirely; if proceeding, document the reasoning.
Gastroparesis or significant GI motility disorder	Generally excluded. Delayed gastric emptying compounds the underlying condition.
Insulin or sulfonylurea therapy	Coordinate with the prescribing physician before initiation. Hypoglycemia risk is real in this group.
Type 1 diabetes	Not a candidate for this program. Refer to endocrinology.
Diabetic retinopathy	Ophthalmology input before initiation; rapid glycemic improvement can worsen retinopathy.
Prior bariatric surgery	Prescriber assessment and coordination with the bariatric team.
Gallbladder disease	Assess and counsel; rapid weight loss increases risk.
Significant renal or hepatic impairment	Prescriber assessment and coordination with the treating physician.
Unstable psychiatric illness	Stabilize and coordinate with the treating clinician before initiating.

4 Disordered Eating Screening

THIS IS THE SCREEN MOST PROGRAMS SKIP

Appetite-suppressing medication given to someone with an active eating disorder is actively harmful. It is also the risk least likely to be documented and most difficult to defend afterwards. Screen every patient, every time, and record it.

Use a validated screening instrument selected and approved by your medical director. Administer at intake and repeat at a defined interval during therapy.

Findings requiring referral before any prescription

- Current or past diagnosis of anorexia nervosa, bulimia nervosa or binge eating disorder.
- Self-induced vomiting, laxative, diuretic or diet pill misuse, current or historical.
- Compulsive exercise, or exercise driven by compensation for eating.
- Rigid food rules, extended fasting, or significant distress around eating.
- Body image concern disproportionate to measured weight.

- A goal weight below a healthy range, or a goal the patient cannot articulate a reason for.
- Family or partner expressing concern about the patient's eating.
- ☐ Screening instrument administered and result recorded at intake.
- ☐ Positive or equivocal screen triggers referral before any prescription is written.
- ☐ Referral pathway to a qualified eating disorder clinician identified and current.
- ☐ Re-screen at the interval set by the medical director during ongoing therapy.
- ☐ New concerns arising during treatment trigger pause and re-referral, not a dose change.

5 Decline & Referral Pathway

- 1 **Explain the reason plainly** and without judgement. Patients decline poorly when they feel dismissed rather than assessed.
- 2 **Offer an alternative** where one exists — referral, co-management with their physician, or reassessment after a specific step.
- 3 **Provide a written referral** where the reason is clinical, addressed to the appropriate specialty.
- 4 **Record the decline:** the criterion not met, the discussion held, the referral made, and the patient's response.
- 5 **Flag the chart** so the same patient is not approved by a different provider next month without the prior assessment being seen.
- 6 **Do not sell an alternative service** as a consolation in the same conversation. It undermines the clinical basis of the decline and reads badly in review.

COMMERCIAL PRESSURE

Weight-loss programs generate recurring revenue and staff may be measured on conversion. State explicitly, in writing and in onboarding, that declining an unsuitable patient is the expected outcome and is never held against anyone. Where that is not culturally true, the screening protocol will not survive contact with a busy Friday.

6 Quick Reference

QUICK REFERENCE CARD

Before the first prescription

1. **Measure** height, weight, BMI, BP, heart rate. No self-report.

2. **MTC / MEN2 question asked and answered.** Recorded verbatim.

3. **Pregnancy, contraception and intent** discussed and documented.

4. **Disordered eating screen** completed. Positive → refer before prescribing.

5. **Insulin or sulfonylurea?** Coordinate with their prescriber first.

6. **Pancreatitis, gastroparesis, retinopathy, bariatric history** reviewed.

7. **Goal agreed in the patient's words** and recorded.

8. **Declining? Document the reason and the referral.**

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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WL-003 Patient Screening & Candidacy — MedSpa Standards v1.0 · January 2026

Page 4 of 4